



Policy

Clinical Supervision in Mental Health

Policy code	EMHS: 49
Original endorsement date	July 2016
Date of current endorsement	October 2024
Version	2
Functional subgroup	Mental Health
Summary	East Metropolitan Health Service encourages all clinical staff and Peer Support Workers to be engaged in a process of clinical supervision. Clinical supervision aims to support the clinician in their professional environment with the goal of improving clinical practice and improving outcomes.
Policy owner	Executive Director Safety, Quality and Consumer Engagement
Contact	
Review date	October 2027
NSQHS Standards (second edition)	Standard 1 Clinical Governance
NSMHS Standards	Standard 8
Impact Statement Declaration (ISD)	The Aboriginal community is reflected in this policy through Aboriginal consultation to ensure cultural appropriateness and positive health impacts on Aboriginal people. (ISD Record ID).
Status	Active

1. Background

1.1. About this document

East Metropolitan Health Service (EMHS) requires all mental health clinical staff, including Aboriginal mental health staff and Peer Support Workers, to be engaged in a process of clinical supervision at a level appropriate to their qualifications and experience.

All EMHS mental health services will have site-based protocols and procedures, to inform and guide their staff in the application of this policy.

1.2. Key definitions

Term	Definition
Clinical supervision	The process of two or more professionals (supervisor and supervisee) meeting formally to reflect on and review clinical situations and practice with the aim to support the clinician in their professional environment. The goal is to further develop knowledge and competence in the context of the clinician's experience and providing safe, high quality, and appropriate care. Clinical supervision provides a formal process of support, reflection, learning and development.
Professional or Clinical Lead	Leads and coordinates the professional discipline and leads the promotion and maintenance of professional standards. Acts as an expert resource and provides consultation, education, and supervision on complex mental health disorders at an advanced practice level to external practitioners and agencies.
Line Manager	The person/manager directly responsible for an employee's performance within the organisation hierarchy.
Supervisee	Any member of each of the professions; medical, allied health and nursing, as well as Peer Support and Aboriginal mental health workers, employed to provide direct clinical care to people accessing mental health services.
Clinical Supervisor	An experienced clinician nominated to discuss clinical work and other professional issues in a structured manner. The supervisor will have the qualifications and skills necessary to supervise the nominated area of clinical practice.

2. Out of Scope

Clinical supervision is one component of professional development and support for EMHS mental health staff engaged in clinical work. For some disciplines clinical supervision can also be a professional board requirement for maintaining registration. Other components of professional development not addressed in this policy include, but are not limited to:

Cultural Consultation

Encourages non-Aboriginal clinicians and staff to maintain proficiency and reflect on their work practice and relationship with Aboriginal people, their carers, families, and communities. It is recommended particularly for non-Aboriginal clinicians who regularly work with Aboriginal people, their carers, families, and communities. Further information about working with Aboriginal people can be found in the [EMHS Aboriginal Cultural Protocols Guidelines](#).

Cultural Supervision

A group session facilitated by a senior Aboriginal clinician that provides Aboriginal mental health workers an opportunity to discuss cultural and clinical concerns in a culturally safe space.

Mentoring

Creating relationships where productive conversations and learning can occur between two colleagues, while providing an opportunity to reflect on career challenges, discuss strategies and celebrate achievements.

Multidisciplinary Team Clinical Reviews

A formal process to monitor clinical assessment and treatment progress and provide an opportunity to identify any barriers to discharge early in the episode of care and plan interventions to address these barriers.

Line Management

The role that the line manager has in operational decision-making and in overseeing the work of the staff member to ensure they are operating within their scope of practice.

Peer Support Worker Peer Supervision

Supervision that is undertaken Peer to Peer. It is described as a safe space to explore strengths, problem-solve challenges and tensions and experience empathy and

validation. Peer supervision keeps Peer Support Workers connected to the underpinning values and principles of the discipline and avoiding drifting into clinical (or other non-lived experience) perspectives and ways of working. Peer supervision may occur internally, externally, or both. Refer to the [Mental Health Commission Lived Experience \(Peer\) Workforces Framework](#).

3. Clinical Supervision in Mental Health

All EMHS mental health clinical staff, including Aboriginal mental health staff and Peer Support Workers, will participate in clinical supervision appropriate to:

- clinical role.
- the requirements of their profession.
- the requirements of the service; and
- statutory and other regulatory requirements.

The purpose of clinical supervision is to:

- provide on-going learning and clinical skill development.
- provide support in coping with the demands of clinical work.
- promote reflective and reflexive practice and maintenance of professional and ethical standards.
- ensure culturally competent care, including Aboriginal cultural responsiveness; and
- promote evidence-based practice.

A clinical supervisor must meet the following minimum standards:

- Have at least two years clinical experience as a mental health professional and be approved by the relevant health board, as is required.
- Preferably be from the same profession as the supervisee.
- Preferably have attained a higher level of skills and experience than the supervisee.
- Have experience and / or training as a clinical supervisor.
- Be available and accessible.

Clinicians and clinical supervisors are required to practice within the process outlined in [Appendix A: EMHS Mental Health Standard of Practice and Procedure for Clinical Supervision](#) when engaging in a clinical supervision arrangement.

3.1. Clinical Supervision for Peer Support Workers

Peer Support Workers will participate in clinical supervision as per the wider clinical team. Clinical supervision for Peer Support Workers should be provided by a clinician with relevant clinical knowledge and skills and who is not the Peer Support Worker's Line Manager. The Supervisor should have previous experience working alongside Lived Experience staff and/or have undertaken education or training to understand the guiding principles for Lived Experience workforces. The Clinical Supervisor is an ally

for the Lived Experience staff and may undertake internal advocacy to support and champion the role.

Clinical Supervisors for Peer Support Workers should be mindful of:

- reducing peer drift – when Peer Support Workers shift towards a medical model of care and deviate from the practices that distinguish them as Peer Support Workers.
- the potential for power imbalances and the need to create mutual, two-way learning relationships through the process of purposeful discussion.

Clinical supervision for Peer Support Workers is different to Peer-to-Peer Supervision, as outlined in the 'Out of Scope' section of this document.

4. Minimum Requirement for Participation in Clinical Supervision

Each staff member who is engaged in providing direct clinical care, or Peer Support services, to people accessing EMHS mental health services is encouraged to participate in a minimum of one (1) hour of formal clinical supervision per month (or pro rata based on FTE) with an identified clinical supervisor. This can occur either individually or within a group environment.

The Line Manager or delegate is to support staff to meet the recommended minimum of one (1) hour clinical supervision per month.

Royal Perth Bentley Group and Armadale Kalamunda Group hub pages provide site specific information regarding the process to request a clinical supervisor.

- [AKG Clinical Supervision hub page](#)
- [RPBG Clinical Supervision hub page](#)

5. Documentation

Both supervisors and supervisees will keep a copy of the documentation related to clinical supervision including a log of the sessions completed.

The supervisee is to maintain a log-sheet of their clinical supervision sessions, which records the date and duration of each session. A copy is to be kept by the supervisor and the log sheet is to be signed by both participants (supervisor and supervisee).

At the time of the supervisee's annual performance development review, the completed log-sheet is to be provided to the supervisee's line manager and professional lead as evidence of their participation in clinical supervision.

Refer to Royal Perth Bentley Group and Armadale Kalamunda Group hub pages for relevant clinical supervision arrangement documentation, Supervision Record Notes and Clinical Supervision Schedule or suitable alternate documentations (e.g., discipline specific documentation)

Clinical supervision arrangement documentation should also include:

- a statement about confidentiality and its limits in relation to clinical supervision and the process for managing inappropriate practice.
- storage and ownership of process notes negotiated between clinician and supervisor within the policy guidelines.

6. Compliance monitoring and legislative penalties

6.1. Compliance monitoring

Each EMHS mental health Site/Service Executive Director or Co-Directors are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

All staff participating in a formal clinical supervision arrangement must:

- provide evidence of their participation to their line manager if requested to do so and at the time of their annual performance development review.
- provide evidence of their participation in any supervision external to the service to their line manager on request, and

Line managers are to keep a record of evidence of staff participation in formal clinical supervision arrangements.

6.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

7. Legislation

- [Mental Health Act 2014](#)
- [Alcohol and Other Drugs Act 1974](#)

8. Additional references

8.1. WA Health Policies

- [Clinical Handover Policy](#)
- [Student Clinical Placement Agreement Policy](#)
- [Clinical Care of People With Mental Health Problems Who May Be At Risk of Becoming Violent or Aggressive Policy](#)
- [Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy](#)

- [State-wide Standardised Clinical Documentation for Mental Health Services](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)

8.2. EMHS Policies

- [Rights of Carers and Other Personal Support Persons](#)
- [Rights of Patients in Mental Health](#)
- [Care Coordination in Mental Health Services](#)
- [Patient Identification and Procedure Matching](#)
- [Consent](#)

8.3. Others

- [Mental Health Commission Lived Experience \(Peer\) Workforces Framework](#)
- [National Practice Standards for the Mental Health Workforce \(2013\)](#)
- [National Standards for Mental Health Services \(2010\)](#)
- [National Safety Quality Health Care Standards \(2021\)](#)
- [Chief Psychiatrists Standards for Clinical Care](#)
- [Australian Health Practitioner Regulation Agency](#)

8.4 Appendix A.

- **Mental Health Standard of Practice and Procedure for Clinical Supervision**

Appendix A: EMHS Mental Health Standard of Practice and Procedure for Clinical Supervision

Standard

Clinical supervision sessions will consist of discussion of the supervisee's clinical work and interactions with people receiving mental health care and will involve consideration of such issues as:

- The nature of the therapy/care being provided.
- The clinical skill utilised to provide the therapy/care.
- Transference and countertransference issues.
- Therapeutic boundaries issues.
- Ethical and professional issues; and
- Cultural, health and well-being need of Aboriginal people, people from ethnoculturally and linguistically diverse backgrounds and people with sexual, gender, and body diversity.

Both supervisors and supervisees will keep a documented record of the process.

Format

Clinical supervision can be provided in many different forms, depending on the clinician's preference and organisational context.

- Individual one to one.
- Group - situation of more than two or more clinicians in an agreed clinical supervision process.
- Cross Discipline - One-to-one or group clinical supervision with more than one professional discipline involved.

Procedure

Establish a Clinical Supervision Arrangement

- Where the clinical supervisor is also the line manager/line supervisor, this relationship is usually outlined in the JDFs of both parties. In such cases, this distinction should be clearly outlined in clinical supervision arrangement documentation, to differentiate between instances where the professional relationship is between line manager and staff member, and where the professional relationship is between supervisor and supervisee.
- It is the responsibility of the Line Manager or delegate to ensure that suitable arrangements for clinical supervision are in place for all staff who report up, directly or indirectly, to their position.

Choose a supervisor.

- Where the clinical supervision relationship is not defined in the JDF, a staff member may prefer to seek a clinical supervision arrangement with another appropriate discipline than their line manager/line supervisor.
- Sites may maintain a register of willing / able Clinical Supervisors to support to identify potential supervisors. Refer to site based clinical supervision hub pages.
- The staff member seeking to set-up a clinical supervision arrangement may approach another mental health professional to provide their clinical supervision. The mental health professional must meet minimum standards of competence as defined below.

A clinical supervisor will:

- Have at least two years clinical experience as a mental health professional and be approved by the relevant health board, as is required.
- Have the qualifications and skills necessary to supervise the nominated area of clinical practice.
- Preferably have attained a higher level of skills and experience than the supervisee.
- Have experience and / or training as a clinical supervisor.
- Be available and accessible.

Approval from Line Manager

- When a supervisor has been found, and they have agreed to provide clinical supervision, the supervisee will approach their line manager/line supervisor for their approval of the arrangement.

Document the Arrangement

- When a supervisor has been found and the approval of the line manager/line supervisor has been obtained, proper documentation of this arrangement must be agreed by both / all parties with each person retaining a copy for their records.

Keep a Record of Sessions

- A record must be kept of the schedule of clinical supervision meetings with all participants retaining a copy for their records.
- The schedule will be kept as a log and will be dated and signed by all participants after each session.
- Supervision session records will be kept confidential and not disclosed for any improper purpose unless disclosure is required by law or HSP Policy to be provided.

Conflict within the Clinical Supervision Arrangement

- If any conflict arises within the clinical supervision arrangement, both parties should attempt to negotiate a mutually acceptable resolution to any issues.
- If issues cannot be resolved satisfactorily this way, then either or both parties should refer the matter to the Line Manager or delegate to assist in resolution of the matter.
- If an agreement cannot be reached between the parties, the endorsed Grievance Resolution Process should be used.

Changing a supervisor

- For a variety of reasons, it may sometimes be necessary or appropriate to change the clinical supervisor.
- Proposed changes to the clinical supervisor/supervision arrangement should be discussed with the Line Manager or delegate in advance and the Line Manager or delegate should approve any such changes.

Supplementing the Primary Clinical Supervision Arrangement

- In certain circumstances it may be appropriate to supplement the primary source of clinical supervision with additional supervision from another party (within the discipline or from another discipline) specific to an area of clinical expertise (e.g., Cognitive Behavioural Therapy). This should be discussed with the line manager or delegate and requires endorsement of the line manager or delegate.

Performance or Risk Issues

- Should major performance or risk issues become apparent during the clinical supervision process, the clinical supervisor should advise the line manager or delegate (if they are not the same person).
- Supervisees should be made aware at the beginning of the supervision arrangement that this may occur as the organisation has a responsibility to ameliorate any risk issues, and the line manager or delegate has the responsibility to constructively manage/resolve any performance or risk issues to achieve a positive outcome.